Case study: Leilani

Condition: **Stroke – right hemisphere stroke with left hemiparesis**

63-year-old woman · Community occupational therapy home visit

Leilani Fonoti is a 63-year-old woman who had a **right hemisphere stroke** resulting in weakness of her left arm and leg. She was discharged home from an inpatient rehabilitation unit four weeks ago. Leilani lives in a low-set house with her husband, Tavita, aged 68, who is her main carer. Their daughter, Mele, lives in Sydney. Before her stroke, Leilani worked part-time as a teacher aide and sang in her church choir for more than 30 years.

At discharge, Leilani's summary documented that she could transfer with standby assistance from one person and walk short distances indoors with a quad stick and supervision. A shower chair and grab rails were recommended, and the community occupational therapy team was asked to check that the equipment had been installed and was suitable.

You arrive for a home visit to check the equipment. Tavita greets you warmly and shows you the bathroom. The grab rails are correctly positioned and firmly fixed, and the shower chair is set at the right height. Leilani is sitting in the lounge room. She greets you with a smile, says "Talofa", and tells you she wants to be back singing with her church choir "before the Christmas concert".

As you talk, Leilani's eyes keep closing. Twice she drifts off mid-sentence and only wakes when Tavita says her name. Her speech is clear when she is awake. Tavita says, "She's just sleepy. She had a good rest last night." When you ask Leilani to show you how she stands up from her chair with her quad stick, she needs far more help than her discharge summary describes, and Tavita takes most of her weight, wincing as he straightens. You notice a large purple bruise on her left forearm and a fresh, uncovered skin tear on her left shin. Leilani says, "I had a little fall." Tavita quietly corrects her: "Three falls this week."

During the visit, Mele phones and Tavita puts her on speakerphone. Mele asks whether her mother can apply for the NDIS because she is under 65, and how long that would take. Tavita then asks whether you could write a letter supporting an application for a disability parking permit, because parking at church is "a nightmare".

When Leilani dozes off again, Tavita follows you into the kitchen. He tells you he hurt his back lifting her off the bathroom floor on Tuesday and has not slept properly for weeks, because she wakes several times each night. "I can't keep doing this," he says, his eyes filling with tears. He shows you a box of his own sleeping tablets, prescribed by his doctor, and explains that he has been giving Leilani "an extra one, sometimes two" at night "so she sleeps through". He says he gave her the last one at about 2 am.

There is no clear documentation of Leilani's current medications, whether she hit her head during any of the falls, or whether her GP knows about the falls. There is also no record of any community supports, respite or carer assessment since her discharge.

As you pack up, Tavita hands you the parking permit form to sign and says he needs to lie down because of his back. He adds that he will "give her half a tablet after lunch so she can have a nap too". Your next home visit is booked in 20 minutes, on the other side of town.

You are the occupational therapy student completing the home visit with Leilani and Tavita and your clinical supervisor is not currently in the room.

Present Leilani's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.

ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

I Introduction

≈ 20 s

Who you are, your role, who the client is and why you need to talk now.

S Situation

≈ 40 s

A brief but complete summary of what is happening and the immediate concern.

B Background

≈ 1 min

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

A Assessment

≈ 2 min 20 s

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

R Recommendation

≈ 2 min 40 s

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

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